

Class 6 – Wrist Fracture: Patient Care Guide

A patient-friendly guide for people with a wrist fracture — a break near the wrist, most often in the lower end of the radius (distal radius) and sometimes the ulna or the small carpal bones such as the scaphoid. It covers causes, symptoms, diagnosis, treatment, healing, exercises, warning signs, and common questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

1. Introduction

A wrist fracture is a break in one of the bones at or near the wrist. The most common type is a distal radius fracture — a break in the lower end of the radius, the larger forearm bone, just above the wrist. It is also called a broken wrist.

Which bones are involved: most often the distal radius; sometimes the lower end of the ulna, and the small carpal bones of the wrist, especially the scaphoid. A Colles' fracture (the broken piece tilts backward) is a very common named pattern.

Common causes: falling onto an outstretched hand, sports injuries, road accidents, and direct blows. How common it is: wrist fractures are among the most common of all fractures, seen frequently in children, active adults, and older adults with osteoporosis.

Who is at higher risk: older adults with weak bones (a wrist fracture is often the first sign of osteoporosis), children, athletes, and people who fall. Severity ranges from an undisplaced crack (mild) to a displaced, joint-involving, or open fracture (severe).

Types of Fractures Affecting This Bone

A wrist fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

Common wrist patterns include Colles' fracture (backward tilt), Smith's fracture (forward tilt), and scaphoid fractures of a small wrist bone, which can be slow to heal because of a limited blood supply.

2. Anatomy

The wrist is a complex joint of forearm and small hand bones, tendons, and nerves.

- Bones involved: the distal radius and ulna (the lower ends of the forearm bones) and the eight small carpal bones, including the scaphoid, that form the wrist.
- Nearby muscles: the forearm flexor and extensor muscles move the wrist and fingers through long tendons that cross the wrist.
- Tendons: many tendons pass across the wrist to move the fingers and thumb; swelling or injury here can limit hand movement.
- Ligaments: strong ligaments hold the carpal bones and the radius and ulna together and keep the wrist stable.
- Blood vessels: the radial and ulnar arteries pass the wrist to supply the hand; the scaphoid has a fragile blood supply, which can slow its healing.
- Nerves: the median nerve runs through the carpal tunnel at the wrist; swelling from a fracture can press on it and cause numbness or tingling in the fingers.

Because the median nerve and many tendons pass through the wrist, a fracture with swelling can cause finger numbness (like carpal tunnel) or stiffness. A distal radius fracture that heals in a poor position can limit wrist and forearm movement, so alignment and exercises matter.

3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For the wrist specifically, a fall onto an outstretched hand (a FOOSH injury) is by far the most common cause.

4. Symptoms

Common signs and symptoms of a wrist fracture include:

- Pain in the wrist, worse with movement or gripping.
- Swelling around the wrist and lower forearm.
- Bruising or discolouration.

- A deformed wrist, sometimes with a 'dinner-fork' shape (Colles' fracture).
- Difficulty moving the wrist or turning the palm.
- Numbness or tingling in the fingers (possible median nerve pressure).
- Reduced grip strength or trouble holding objects.
- Tenderness in the 'anatomical snuffbox' at the thumb base (possible scaphoid fracture).
- In an open fracture, bone may be visible through a wound.

Emergency warning signs: a cold, pale, or blue hand, spreading or severe finger numbness, uncontrolled bleeding, a severely deformed wrist, or bone through the skin. Seek emergency care immediately.

5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For a wrist fracture, X-rays of the wrist from more than one angle are the main test. A CT scan may be used for complex or joint-surface fractures before surgery. An MRI or bone scan is often used for a suspected scaphoid fracture, because it may not show on early X-rays but needs treatment to avoid healing problems.

6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

Non-Surgical Treatment

Many wrist fractures, especially undisplaced ones or those that can be gently set, heal well without surgery.

- **Cast:** a plaster or fibreglass cast (usually below the elbow) holds the wrist while it heals, commonly for several weeks. A scaphoid fracture may need a thumb-including cast.
- **Splint:** used first, especially with swelling, before a full cast.
- **Closed reduction:** for a displaced fracture, the doctor gently realigns the bone (often with local anaesthetic) before applying the cast.
- **Brace:** a removable wrist brace may be used in later healing for some fractures.
- **Ice therapy:** reduces pain and swelling in the first days.
- **Elevation:** keeping the hand raised above heart level reduces swelling.
- **Pain medications:** paracetamol for pain relief.
- **Anti-inflammatory medicines:** such as ibuprofen, if approved by your doctor.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

Surgical Treatment

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- **ORIF (Open Reduction Internal Fixation):** the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- **Plates:** flat metal supports screwed along the bone to hold the pieces together.
- **Screws:** hold fragments together, alone or with a plate.
- **Pins (K-wires):** thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- **Rods (intramedullary nails):** metal rods placed inside the hollow centre of a long bone to support it.
- **External fixator:** a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- **Bone graft:** extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): displaced or unstable fractures, fractures involving the wrist joint surface, fractures that slip out of position in a cast, open fractures, and displaced scaphoid fractures. The distal radius is often fixed with a plate and screws; some fractures use pins or an external fixator, and scaphoid fractures may be fixed with a special screw.

Recovery Expectations and Risks After Surgery

- **Recovery:** a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- **Risks:** infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- **Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.**

7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

Group	Typical healing / recovery
Children	Often 4–6 weeks to unite; children's wrists heal and remodel well.
Adults	Distal radius about 6–8 weeks; scaphoid often 8–12 weeks or longer.
Older adults	8 weeks or more; healing slower with osteoporosis.
Athletes	Union in 6–8 weeks; return to sport often 8–12 weeks with protection.

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Regaining full wrist bending and forearm rotation, plus grip strength, is the main functional goal.

- Return to work: desk work in 1–2 weeks; heavy manual work often 8–12 weeks or after clearance.
- Return to sports: non-contact around 8 weeks; contact sports usually 10–12 weeks with a guard.
- Full recovery: 3–6 months for full grip and movement; scaphoid fractures can take longer.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

Stage	What usually happens
Week 1	Most pain and swelling; hand kept raised; move fingers, elbow, and shoulder.
Week 2	Pain settling; swelling reducing; keep fingers and thumb moving fully.
Week 3–4	Cast continues; bone knitting; finger stiffness is common — keep them moving.
Week 5–8	Bone uniting; cast often removed toward the end; gentle wrist movement begins.
Month 3	Bone usually united; physiotherapy for wrist movement and grip strength.
Month 6	Most grip and movement restored; heavier tasks resumed.

9. Daily Care Instructions

- Keep the cast dry: cover it with a plastic bag while bathing; a wet cast weakens and irritates the skin.
- Sleeping position: rest the hand on a pillow, slightly raised, to reduce night swelling; avoid lying on the injured wrist.
- Bathing: use a waterproof cover or sponge-bathe; dry the cast edges if damp.
- Clothing: wear loose sleeves; remove rings early because swelling can trap them.

- Walking: unaffected — normal walking is fine; a sling adds comfort early on.
- Driving: avoid driving while in a wrist cast or when you cannot grip and turn the wheel safely.
- Returning to office: light desk work is often possible within days to 1–2 weeks.
- Returning to school: children can usually attend, avoiding sport and rough play.
- Household activities: avoid gripping, lifting, wringing, and weight-bearing on the wrist.

10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

Key Nutrients for Bone Healing

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Calcium	Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults.	Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds	Small fish eaten with bones (sardines), dairy
Vitamin D	Helps the body absorb calcium. About 600–800 IU/day (more if deficient).	Sunlight, fortified milk and cereals, mushrooms	Egg yolk, fatty fish (salmon, mackerel), fish liver oil
Protein	Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.	Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts	Eggs, chicken, fish, lean meat

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Vitamin C	Needed to make collagen for bone repair. About 65–90 mg/day.	Oranges, lemon, amla, guava, capsicum, tomato, broccoli	(Mainly from plant foods)
Vitamin K	Helps bind calcium into bone. About 90–120 mcg/day.	Spinach, kale, broccoli, cabbage, green leafy vegetables	Eggs, some dairy
Magnesium	Supports bone structure and vitamin D use. About 310–420 mg/day.	Nuts, seeds, whole grains, bananas, dark leafy greens	Fish
Phosphorus	Works with calcium to harden bone. About 700 mg/day.	Dairy, beans, lentils, whole grains, nuts	Meat, fish, poultry, eggs
Zinc	Helps bone cells grow and repair. About 8–11 mg/day.	Whole grains, legumes, seeds, nuts	Meat, shellfish, eggs
Omega-3 fats	Reduce inflammation and support healing.	Flaxseed, chia seeds, walnuts, soybean oil	Fatty fish (salmon, mackerel, sardines)
Collagen	Forms the flexible base of bone. Supported by protein + vitamin C.	Body makes it from protein-rich plant foods + vitamin C	Bone broth, chicken, fish, eggs

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will tailor them to you.

Early Stage

- Finger, thumb, elbow, and shoulder movements while in the cast — purpose: prevent stiffness and swelling. Repetitions: gentle, 10 each, several times daily. Precaution: keep the wrist still.
- Making a gentle fist and opening the hand — purpose: keep tendons gliding and reduce swelling.

Middle Stage

- Wrist bending up and down (after cast removal) — purpose: restore wrist movement. Repetitions: 10 each way, 2–3 times daily. Precaution: gentle stretch only.
- Forearm rotation (turning palm up and down) — purpose: regain rotation. Repetitions: 10 slow turns.
- Side-to-side wrist movements — purpose: restore full wrist motion.

Advanced Stage

- Grip strengthening with a soft ball or putty — purpose: rebuild grip. Repetitions: 10–15 squeezes, 2–3 sets. Precaution: start light.
- Resistance-band wrist and forearm exercises — purpose: build strength and endurance.
- Weight-bearing and load progression (for example, gentle press-ups against a wall) — purpose: prepare for full use. Precaution: increase slowly; stop if pain returns.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

Activity	How long to avoid (typical)
Gripping and lifting with the injured wrist	Until healed — often 6–8 weeks
Weight-bearing on the hand (press-ups, planks)	About 3 months or until cleared
Weight training that loads the wrist	About 3 months or until cleared
Contact sports (cricket, football, skating)	About 10–12 weeks, then use a wrist guard
Carrying heavy bags in that hand	Until full strength returns (about 2–3 months)
Driving with the wrist in a cast	Until the cast is off and grip is safe

15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.
- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.
- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

Visit	What the doctor usually checks
Week 1	Check swelling, cast fit, finger movement, feeling, and position on X-ray.
Week 2	Confirm the bone has not shifted (especially after reduction); adjust cast if needed.
Week 4	X-ray for early healing; check finger stiffness and pain.
Week 6	Check union; the cast may be removed or changed to a brace; begin wrist movement.
Week 12	Confirm full union (longer follow-up for scaphoid); review grip, movement, and return to activity.

18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

Activity	Typical time to return
School	A few days to 1 week (no sport/rough play)
Office (desk job)	A few days to 2 weeks
Driving	After the cast is off and grip is safe — often 6–8 weeks
Gym (upper body)	About 3 months, avoiding wrist-loading early
Swimming	After the cast is off and skin has healed — about 8 weeks
Cycling	Light cycling around 8 weeks (safe grip needed; avoid falls)
Running	Usually within 2–4 weeks (does not load the wrist)
Cricket / Football	About 10–12 weeks, often with a wrist guard
Weight lifting	About 3 months, increasing wrist load slowly

19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.

- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.
- Sports safety: warm up, use correct technique, and wear the right gear.
- Fall prevention: good footwear, adequate lighting, and vision checks.
- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.
- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

20. Frequently Asked Questions (FAQ)

Q1. Can I move my fingers?

Yes, and you should. Gently move your fingers, thumb, elbow, and shoulder while in the cast to prevent stiffness and reduce swelling. Keep the wrist itself still.

Q2. Can I sleep on the injured wrist?

No. Rest the hand on a pillow, slightly raised, to reduce swelling and pain. Avoid lying on the injured wrist.

Q3. Can I remove the cast or splint?

No. Never remove or cut your cast yourself. It must stay on until your doctor decides the bone has healed enough.

Q4. When can I drive?

Usually only after the cast is off and you can grip and turn the wheel safely — often around 6–8 weeks. Confirm with your doctor.

Q5. Can I exercise?

Yes, lower-body and cardio exercise is usually fine. Avoid wrist-loading and gripping exercises until cleared.

Q6. Can I lift weights?

Not until the bone heals and grip is pain-free — usually about 3 months, starting light.

Q7. Is swelling normal?

Some swelling is normal in the first days to weeks. Keep the hand raised and use ice. Report sudden, severe, or increasing swelling, or a cold/blue hand, urgently.

Q8. Is itching inside the cast normal?

Yes, mild itching is common. Do not push objects inside to scratch, as this can injure the skin. Cool air can help.

Q9. Can I travel?

Short travel is usually fine once pain is controlled. Keep the hand raised. Ask your doctor before long journeys.

Q10. Can I fly?

Air travel is generally allowed; tell your doctor. Soon after a new cast, some doctors advise a split cast because swelling can increase at altitude.

Q11. Can I get the cast wet?

No. Keep a standard plaster or fibreglass cast dry with a waterproof cover during bathing to protect the skin and cast.

Q12. How long should I wear the cast?

A distal radius fracture is often casted about 4–6 weeks; a scaphoid fracture may need 8–12 weeks or longer.

Q13. When should I see a doctor urgently?

For a cold or blue hand, spreading finger numbness, severe pain, a very tight or broken cast, fever, or discharge from the cast.

Q14. Why do my fingers feel numb or tingly?

Swelling can press on the median nerve at the wrist, causing carpal-tunnel-like numbness. Keep the hand raised and tell your doctor if it persists or worsens.

Q15. Why is my wrist so stiff after the cast?

Weeks in a cast make the wrist stiff. This is common and improves with physiotherapy over several weeks.

Q16. Will I need surgery?

Not always. Many wrist fractures heal in a cast. Surgery is used for displaced, unstable, joint, or open fractures, and displaced scaphoid fractures.

Q17. Do the plates or screws need removal?

Usually they can stay for life and are removed only if they cause pain or irritation.

Q18. What is a scaphoid fracture and why is it special?

The scaphoid is a small wrist bone with a fragile blood supply, so it can heal slowly or fail to heal. It often needs a longer cast or a screw, and careful follow-up.

Q19. Is some pain normal weeks later?

Mild aching during healing and physiotherapy is normal. Sharp or worsening pain should be reviewed.

Q20. What foods help my wrist heal?

Calcium, vitamin D, protein, and vitamin C support bone repair. Avoid smoking and alcohol, which slow healing.

Q21. Can I go to work?

Desk work is often possible within days to two weeks. Heavy manual work usually needs 8–12 weeks or clearance.

Q22. Will my wrist be weak after the cast comes off?

Yes, temporary weakness and stiffness are normal. Physiotherapy and grip exercises restore strength and movement.

Q23. Can I type or use my phone?

Light typing and texting with the fingers is usually fine if it does not hurt and the wrist stays supported in the cast.

Q24. Can I play sports again?

Yes, most people do. Non-contact around 8 weeks and contact sports around 10–12 weeks, often with a wrist guard, after clearance.

Q25. How can I prevent another wrist fracture?

Strengthen bones with exercise, get enough calcium and vitamin D, prevent falls, wear wrist guards in sport and skating, and get osteoporosis screening if you are at risk — a wrist fracture can be an early warning.

21. Metadata (for Retrieval)

Structured tags to help the AI assistant retrieve this document accurately.

Field	Value
Fracture Name	Wrist Fracture
Bones Involved	Distal radius, distal ulna, carpal bones (especially scaphoid)
Body Region	Upper limb (wrist)
Common Causes	Fall onto outstretched hand (FOOSH), sports, road accidents, direct trauma, osteoporosis
Symptoms	Pain, swelling, bruising, dinner-fork deformity, stiffness, finger numbness, weak grip
Treatment Options	Cast, splint, closed reduction, brace; ORIF with plate/screws, pins, external fixator, scaphoid screw
Healing Duration	Distal radius 6–8 weeks; scaphoid 8–12+ weeks; full recovery 3–6 months
Rehabilitation Duration	6–16 weeks of physiotherapy; grip strength returns gradually
Emergency Signs	Cold/blue hand, spreading numbness, tight cast, open bone, compartment syndrome
Keywords	wrist fracture, broken wrist, distal radius fracture, Colles fracture, Smith fracture, scaphoid fracture, FOOSH
Synonyms	Broken wrist, cracked wrist bone, distal radius break
Related Injuries	Forearm fracture, finger fracture, carpal tunnel symptoms, scaphoid nonunion